

Transport Decision Process

Purpose

To define a process for transferring care of patients to providers with credentialing levels $\leq$ PL-4.

Application:

In this standard, a $\geq$ PL-5 refers to an ATCEMS credentialed $\geq$ PL-5 with no current restrictions on their credential to practice.

All providers on scene are expected to participate in patient care; the highest credentialed provider on scene maintains ultimate responsibility for the patient's care and for ensuring that the patient is adequately cared for en route to the destination. Stable patients not in need of $\geq$ PL-5 level care may be attended by another provider. A $\geq$ PL5 must manage any "High-Risk Transport" patient as defined below:

1. Trauma Alert (Physiologic criteria)
2. Stroke Alert
3. LVO Alert
4. STEMI alert
5. Patients who have experienced syncope
Exception: Patients <50 years of age who return to normal baseline mental status, have a normal ECG, do not have a complaint that otherwise meets **High-Risk Transport** criteria, and no **vital signs** meet high-risk refusal criteria.
6. Postictal seizure patients who have not returned to baseline mental status
7. Patients who require additional or ongoing medication(s), intervention(s), and/or monitoring beyond the scope of the $\leq$ PL-4 as defined by the clinical operating guidelines.
8. Patients who have received medications beyond the scope of practice of the $\leq$ PL-4 provider.
Exceptions:
 1. Patients who receive a single dose of Intranasal narcotic for pain control of a traumatic injury that does not involve the head, neck, chest, or abdomen.
 2. Patients with only an IV saline lock
 3. Patients receiving only monitored PO medications
 4. Initially hypoglycemic patients who return to normal baseline status after treatment and have no **vital signs** that meet high-risk refusal criteria
9. Patients who the on-scene providers do not agree can be safely transported without a $\geq$ PL-5 attending in the patient compartment.

BLS Unit Application:

Any "**High-Risk Transport**" patient as defined above must be assessed by a $\geq$ PL-5 provider or responder, if available. However, if a BLS unit has been dispatched to the scene and on-scene providers determine that, although the patient meets the criteria for PL-5 evaluation/care, the patient would receive significant clinical benefit from rapid transport to the nearest appropriate facility and/or the patient would be significantly harmed by waiting on scene for a $\geq$ PL-5, the BLS unit may transport the patient to the nearest appropriate facility.

Documentation:

The ePCR should reflect the decision-making process to determine which provider attends to the patient.